

SAME SIZE ARTWORK
LEAFLET SIZE: 140 mm x 88 mm

DRUG INTERACTIONS

None reported.

PRECAUTIONS / WARNINGS

CANDID-B Cream is normally well tolerated. If irritation develops, the preparation should be discontinued. This formulation should not be used in the eyes or in the ear if the eardrum is perforated.

Though Topical steroids have not been reported to cause adverse effect on pregnancy, their safety in pregnancy has not been firmly established. They should, therefore, not be used in large amounts or for long periods of time during pregnancy.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION

The affected area is washed and dried. **CANDID-B** Cream is then rubbed in. In hairy sites, the hair should be parted before application to allow direct contact with the lesions. The frequency of application is generally 2 to 3 times daily. After controlling the eczematous symptoms, a switch over to **CANDID** Cream or Lotion may be advised till the primary fungal infection is completely eradicated.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE AND ANTIDOTE (S)

Not applicable since this formulation is for topical use and are normally well tolerated. However, if irritation develops, the preparation should be discontinued.

PACKING/ PACK SIZES

Tube of 5g, 15g & 60g.

SHELF LIFE

3 Years

STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICAL PRECAUTIONS

Store below 30°C. Protect from light.

MAL 19872019AZ

Date of Revision: 1 October 2018

For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

candid-BTM
Cream

SPACE FOR
PHARMACODE

COMPOSITION

Clotrimazole USP 1%
Anhydrous Beclometasone Dipropionate BP 0.025 %

Preservatives:

Methyl Paraben 0.15% w/w
Propyl Paraben 0.05% w/w

THERAPEUTIC CLASS

T0130

DESCRIPTION

A printed carton with an insert and a printed aluminium collapsible tube containing a white semi solid cream.

ACTIONS AND MODE OR MECHANISM OF ACTION

CANDID-B Cream is a broad spectrum Antimycotic, Anti-inflammatory and Antipruritic. It offers effective therapy for dermatomycoses complicated by inflammation, pruritus, erythema or other eczematous features.

The active ingredients of **CANDID-B** Cream are Clotrimazole and Beclometasone Dipropionate.

Clotrimazole has broad spectrum fungicidal activity against Dermatophytes (Trichophyton, Epidermophyton and Microsporum spp.), Candida and Aspergillus spp.,

Manufactured by:
glenmark
PHARMACEUTICALS LTD.
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Product Registration Holder
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Malassezia furfur and other fungi. It also inhibits some strains of Gram-positive and Gram-negative bacteria. Beclometasone Dipropionate, the potent and fast acting topical corticosteroids exerts marked anti-inflammatory, antipruritic, anti-allergic and vaso-constrictive effects. On account of these properties it gives prompt relief in inflammation, pruritis, erythema etc.

PHARMACOLOGY (SUMMARY OF PHARMACODYNAMICS AND PHARMACOKINETICS)

CANDID-B Cream is a combination of Clotrimazole and Beclometasone Dipropionate. The pharmacological aspects of both the ingredients are given below:

Clotrimazole

Clotrimazole is an imidazole antifungal agent and is effective against a wide variety of fungi including Epidermophyton, Microsporum and Trichophyton spp., Coccidioides immitis, Histoplasma capsulatum, Aspergillus and Candida spp. Other fungi responding to Clotrimazole include Cryptococcus neoformans, Malassezia furfur (Pityrosporum orbiculare), Paracoccidioides brasiliensis, Sporothrix schenckii, Pseudallescheria, Medurella and Phialophora spp.

Clotrimazole also has antibacterial action and some strains of Staphylococcus aureus and Streptococcus pyogenes have been reported to be sensitive.

Clotrimazole is believed to exert its activity on the cell membrane of the fungus. It appears that the drug inhibits protein synthesis and so influences the utilisation and the incorporation of the acylated amino acids essential for fungi.

Beclometasone Dipropionate

Beclometasone Dipropionate is a synthetic glucocorticoid and is used by topical application for the treatment of various dermatoses.

The topical application of Beclometasone Dipropionate produces dramatic suppression of dermatoses by potent and rapid anti-inflammatory, anti-pruritic, anti-allergic, anti-proliferative and vaso-constrictive actions.

Unlike fluorinated topical steroids, Beclometasone Dipropionate is less likely to produce adrenal suppression or changes in the skin as Beclometasone Dipropionate is metabolised in the skin.

PHARMACOKINETICS

Clotrimazole appears to be well absorbed in humans following oral administration and is eliminated mainly as inactive metabolites. Following topical and vaginal administration, however, Clotrimazole appears to be minimally absorbed. Six hours after the application of radioactive Clotrimazole 1% cream and 1% solution into intact and acutely inflamed skin, the concentration of Clotrimazole varied from 100 mcg / cm³ in the stratum corneum to 0.5 to 1 mcg/cm³ in the stratum reticulare, and 0.1 mcg/cm³ in the subcutis. No measurable amount of radioactivity (≤ 0.001 mcg/ml) was found in the serum within 48 hours after application under occlusive dressing of 0.5 ml of the solution or 0.8 g of the cream. Only 0.5% or less of the applied radioactivity was excreted in the urine.

The extent of percutaneous absorption of topical corticosteroids is determined by many factors including the vehicle, the integrity of the epidermal barrier, and the use of occlusive dressings. Topical corticosteroids can be absorbed from normal intact skin.

Inflammation and/or other disease processes in the skin increases percutaneous absorption. Occlusive dressings substantially increase the percutaneous absorption of topical corticosteroids.

Once absorbed through the skin, topical corticosteroids are handled through pharmacokinetic pathways similar to systemically administered corticosteroids. Corticosteroids are bound to plasma proteins in varying degrees. Corticosteroids are metabolised primarily in the liver and are then excreted by the kidneys. Some of the topical corticosteroids and their metabolites are also excreted into the bile.

TOXICOLOGY

Since **CANDID-B** Cream is for topical use only and does not enter systemic circulation and the drug restricted to the skin layers by the normal dermal barrier, there is no evidence of systemic toxicity in both humans and animals.

As **CANDID-B** Cream contains potent corticosteroid long term continuous therapy with the product should be avoided particularly in infants and children in whom adrenal suppression may occur. The weekly dosage is generally restricted to less than 50 g for adults and less than 25 g for children. If **CANDID-B** Cream is applied to large areas of the body (10% and more) and for long periods of time (more than 4 weeks) particularly when occlusive dressings are used, local symptoms such as atrophy of the skin, telangiectasia, striae and acneiform changes of the skin and systemic effects of the corticosteroids may be seen.

INDICATIONS

CANDID-B Cream can be of benefit in the fungal infections of the skin, particularly when accompanied by eczematous features:

- Tinea Capitis
- Tinea Corporis
- Tinea Barbae
- Tinea Cruris
- Tinea Manum
- Tinea Unguium
- Tinea Pedis
- Tinea Versicolor
- Erythrasma
- Vulvo-Vaginal Candidiasis
- Candidal Nappy Rash
- Candidal Paronychia

CONTRAINDICATIONS

These formulations should not be used in tuberculosis of the skin, herpes simplex, chicken pox, measles, vaccinia, syphilitic skin lesions and in cases of hypersensitivity to any of the components.

SIDE EFFECTS / ADVERSE REACTIONS

The use of steroids topically over large skin areas, with or without occlusive dressing, may result in systemic absorption of the steroid and suitable precautions should be taken by physician. When occlusive dressings are put, miliaria, folliculitis or pyoderma may sometimes develop beneath the occlusive material. Localised atrophy and striae have been reported with topical steroids.

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ICONGRAPHICS CODE:

PANTONE SHADE